LixiLan-L Trial (Aroda et al., 2016) — Published Abstract

Aroda VR, Rosenstock J, Wysham C, et al. Efficacy and Safety of LixiLan, a Titratable Fixed-Ratio Combination of Insulin Glargine Plus Lixisenatide in Type 2 Diabetes Inadequately Controlled on Basal Insulin and Metformin: The LixiLan-L Randomized Trial. *Diabetes Care.* 2016;39(11):1972–1980. DOI: 10.2337/dc16-1495 · PubMed: 27650977

Note for candidates: The full text of this paper is behind a paywall. Reproduced below is the **publicly available abstract** from the journal — this is the source you should use to verify claim **C5 (weight)**. Use the numbers reported here (and the weight charts on the SOLIQUA LBL itself) for your verification.

CLINICAL CARE/EDUCATION/NUTRITION/PSYCHOSOCIAL RESEARCH | SEPTEMBER 20 2016

Efficacy and Safety of LixiLan, a Titratable Fixed-Ratio Combination of Insulin Glargine Plus Lixisenatide in Type 2 Diabetes Inadequately Controlled on Basal Insulin and Metformin: The LixiLan-L Randomized Trial

Vanita R. Aroda; Julio Rosenstock; Carol Wysham; Jeffrey Unger; Diego Bellido; Guillermo González-Gálvez; Akane Takami; Hailing Guo; Elisabeth Niemoeller; Elisabeth Souhami; Richard M. Bergenstal; on behalf of the LixiLan-L Trial Investigators

Check for updates

Corresponding author: Vanita R. Aroda, vanita.aroda@medstar.net.

* A complete list of the LixiLan-L principal investigators can be found in the Supplementary Data online.

Diabetes Care 2016;39(11):1972–1980

https://doi.org/10.2337/dc16-1495 **Article history**

PubMed:27650977

Connected Content

A correction has been published: Erratum. Efficacy and Safety of LixiLan, a Titratable Fixed-Ratio Combination of Insulin Glargine Plus Lixisenatide in Type 2 Diabetes Inadequately Controlled on Basal Insulin and Metformin: The LixiLan-L Randomized Trial. Diabetes Care 2016;39:1972–1980

A companion article has been published: Benefits of LixiLan, a Titratable Fixed-Ratio Combination of Insulin Glargine Plus Lixisenatide, Versus Insulin Glargine and Lixisenatide Monocomponents in Type 2 Diabetes Inadequately Controlled on Oral Agents: The LixiLan-O Randomized Trial

Share Cite Get Permissions

OBJECTIVE

OBJECTIVE

This study was conducted to demonstrate the efficacy and safety of LixiLan (iGlarLixi), a novel, titratable, fixed-ratio combination of insulin glargine (iGlar) (100 units) and lixisenatide, compared with iGlar in patients with type 2 diabetes inadequately controlled on basal insulin with or without up to two oral glucose-lowering agents.

RESEARCH DESIGN AND METHODS

After a 6-week run-in when iGlar was introduced and/or further titrated, and oral antidiabetic drugs other than metformin were stopped, 736 basal insulin-treated patients (mean diabetes duration 12 years, BMI 31 kg/m^2) were randomized 1:1 to open-label, once-daily iGlarLixi or iGlar, both titrated to fasting plasma glucose <100 mg/dL (<5.6 mmol/L) up to a maximum dose of 60 units/day. The primary outcome was change in HbA_{1c} levels at 30 weeks.

RESULTS

HbA_{1c} decreased from 8.5% (69 mmol/mol) to 8.1% (65 mmol/mol) during the run-in period. After randomization, iGlarLixi showed greater reductions in HbA_{1c} from baseline compared with iGlar (–1.1% vs. –0.6%, $P < 0.0001$), reaching a mean final HbA_{1c} of 6.9% (52 mmol/mol) compared with 7.5% (58 mmol/mol) for iGlar. HbA_{1c} <7.0% (53 mmol/mol) was achieved in 55% of iGlarLixi patients compared with 30% on iGlar. Mean body weight decreased by 0.7 kg with iGlarLixi and increased by 0.7 kg with iGlar (1.4 kg difference, $P < 0.0001$). Documented symptomatic hypoglycemia (≤70 mg/dL) was comparable between groups. Mild gastrointestinal adverse effects were very low but more frequent with iGlarLixi.

CONCLUSIONS

Compared with iGlar, a substantially higher proportion of iGlarLixi-treated patients achieved glycemic targets with a beneficial effect on body weight, no additional risk of hypoglycemia, and low levels of gastrointestinal adverse effects in inadequately controlled, basal insulin-treated, long-standing type 2 diabetes.

GoNuts.ai · Fractiva — Citation reference (C5) · Abstract reproduced from Diabetes Care for assessment use · Confidential